



Mental Health Patient Management within the Emergency Department SOP

Scope

Site	Department, Division or Operational Area	Applicable to
Royal Perth Hospital (RPH) only	Emergency Department (ED), Mental Health (MH) Division	Medical, Nursing and Allied Health Staff

Purpose

This standard operational procedure (SOP) outlines the best practice standards and processes for the triage, assessment, treatment and discharge of patients who present to the ED with suspected or confirmed mental illness.

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Background

Timely MH assessment and care can limit prolonged admission and/or inappropriate discharge into the community. The ED Mental Health Liaison Team (MHLT) aim to:

- Provide timely assessment and contribute to the patient's care, management plan and support of carers and family.
- Improve patient flow in ED by establishing and/or supporting care pathways for transition to a discharge destination, e.g. MH units, home, or other, appropriate internal and/or external health services.
- Promote integration between ED and RPBG MH services and/or external stakeholders by rapidly addressing presenting needs and planning the patient's ongoing care.
- Promote evidence-based care to patients and support persons that is:
 - Recovery orientated, promoting a sense of hope and recovery.
 - Person centred.
 - Trauma informed.
 - Culturally competent for Aboriginal patients and their families and for the particular needs of diverse and specific populations.
 - Suitable to patient's developmental needs.
- As per the [Health Department WA \(HDWA\) Safety Planning for Mental Health Consumers Policy](#).

The ED MHLT comprises of senior (onsite) MH clinicians who provide a 24-hour / 7-day multidisciplinary (MDT) in-reach service to ED clinicians to support provision of care to MH patients and to specifically, facilitate leadership with patient flow and management of acute events.

For MH patients, care provision will be undertaken in the context of the Mental Health Act (MHA) 2014 and the [Charter of Mental Health Care Principles \(MHA 2014\)](#).

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Definitions

Mental health condition	Disorder affecting a patient's mood, thinking and behaviour.
Absence without leave (AWOL)	Patient is absent without leave approved by a Medical Practitioner from a hospital, service or other place.
Missing person	Patient who has left the care of the hospital or service without prior knowledge, notification or agreement or, in the case of involuntary patients, authorisation. The patient is a 'missing person' regardless of their MH, legal or leave status.
Psychiatric Online Service Information System (PSOLIS/ webPSOLIS)	Patient database used by WA public Mental Health services.
Authorised Mental Health Practitioner (AMHP)	A MH clinician deemed by the Chief Psychiatrist to have the qualifications, training and experience appropriate to perform the functions of the role under the MHA 2014. E.g. a Psychologist, Nurse, Occupational Therapist or Social Worker with at least 3 years' experience in the management of patients who experience mental illness.
Emergency Psychiatric Treatment (EPT)	Emergency psychiatric treatment is treatment that needs to be provided to a person: to save the person's life; or to prevent the person from behaving in a way that is likely to result in serious physical injury to the person or another person (s202(1)).
State-wide Standardised Clinical Documentation (SSCD)	Indicates the minimum, mandatory suite of clinical documentation for MH clinicians to complete at presentation, once admitted and when a patient is discharged. Refer to the HDWA State-wide Standardised Clinical Documentation for Mental Health Services Policy .
Royal Perth Bentley Group (RPG) Community Mental Health Service (CMHS)	Comprising of MDT teams that provide short (up to 10 weeks), medium and long term (over 10 weeks) recovery-focussed mental health care in the community for youth, adults (aged 18 - 64 years) and older adults (65 years+) with a mental illness. Specifically, services are located in the following catchment areas: Youth (Bentley), adults and older adults in Bentley, Midland and City East
Support Person	Includes personal support person (PSP), next of kin (NoK), family, carer, friend, significant other, an Office of the Public Advocate guardian, enduring guardian or a nominated person

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Procedure

- The ED staff and ED MHLT will collaboratively assess and manage patients presenting to the ED with MH presentation
- Medical treatment does not need to be completed prior to review by the ED MHLT. Concurrent assessment and management are encouraged.
- Patients will be referred to the ED MHLT via Computer Assisted Radio Personnel System (CARPS).
- If further information required, ED MHLT to contact referring clinician.
- An Enterprise Bed Management (EBM) bed request will be submitted
 - If further information is required, ED MHLT to contact referring clinician.
- The ED clinician will clearly communicate the nature of the referral request. E.g., patient for assessment, a check of the history for a known or suspected MH patient, advice.
- Referrals are to be acknowledged by ED MHLT within 30 minutes.
 - If ED MHLT are unable to commence assessment within one hour due to the volume of assessments to be completed, discussion with Emergency Physician in Charge (EPIC) is expected to form an agreed prioritisation of patients.
- When a patient presents to ED with a MH related issue or complaint, review PSOLIS/ web PSOLIS to confirm if patient is active with community mental health team.
 - The patient must, at minimum, be discussed with the ED MHLT prior to discharge.
- Clinicians are to ensure that all MH patients presenting to the ED have a medical assessment.
 - The involvement of the Clinical Nurse Specialist Mental Health Liaison (CNS MHL) is not a substitute for this.
- Patients presenting to the ED with a primary MH complaint can be referred directly to the ED MHLT by the ED Triage Nurse if they:
 - Meet the criteria on the Provisional Primary Mental Health Streaming (PPMHS) Tool
 - Are triaged as the Australian Triage Scale (ATS) 4 and 5 – refer to the [RPBG ED Triage and Waiting Room SOP](#).

Governance

All patients are under the governance of the ED until referred to and assessed by the ED MHLT. Once this occurs the patient becomes under the governance of the ED MHLT who will make a decision as to whether the patient is required to be admitted, discharged, or on-referred (see [RPBG Inpatient Admissions from the ED Policy](#)). Whilst a patient remains in the ED, the ED medical staff retain treating responsibility for any immediate medical concern (see [RPBG ED Deteriorating Patient Escalation SOP](#))

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Roles and Responsibilities

RPBG Community Mental Health Service (CMHS)

When CMHS clinical staff determine that a community MH patient requires admission and there are no direct ward or service beds available, or the patient is not suitable for a direct admission (refer to the [RPBG General Health Admission, Discharge and Transfer SOP](#) and [RPBG Mental Health Inpatient Triage to Discharge SOP](#)), the clinician will:

- Inform EMHS Mental Health Patient Flow Coordinator (in hours) 0700-1700 or Operations Hub Clinical Nurse Manager (CNM) after hours of the need for patient transfer.
- Provide a direct telephone handover to EPIC, ideally medical to medical.
 - Occasionally, this may be provided by AMHP.
- Phone the CNS MHL to discuss the reason for presentation, plan and estimated time of arrival (ETA).
- Complete a Mental State Examination (MSE) and plan for ED attendance via the SSCD Mental Health Assessment Form (MHAF) or Triage form (where appropriate) and Risk Assessment Management Plan (RAMP) in PSOLIS/ webPSOLIS.

Refer to [Community Mental Health Services Triage to Discharge SOP](#).

Support people and/or external agencies

- The vital role of a patient's support person(s) to provide collateral information regarding the patient's mental state will be recognised and respected.
- The MHLT will identify and contact the patient's support person(s) wherever possible to obtain collateral history and support them to participate in assessment and treatment planning.
- Collateral information may include early warning signs of change in the persons' MH, management strategies that have worked in the past and the impact of medication.
- Wherever possible, a patient's support person(s) will be actively included in safety and discharge planning.
- Support persons of patients who have made a suicide attempt or are experiencing thoughts of suicide are to be provided information regarding:
 - how to escalate care if the patient deteriorates.
 - safety planning.
 - referrals and aftercare.
 - available MH crisis services.
- Support person(s) are to be informed of relevant notifiable events.
- All communication with the patients' support person(s) is to be documented in the patient's healthcare record.
- Crisis care workers provided by the Department of Communities provide emotional and practical support to patients less than 16 years of age; similar to the role of support people.
- Support person(s) and external agencies have no responsibility for the clinical supervision of the patient in the ED.

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Clinical Nurse Specialist Mental Health Liaison (CNS MHL) (24/7)

- Is the main point of contact, communication, and coordination between the ED MHLT and the ED team.
- Reviews all referrals from the ED team within 30 minutes or liaises with EPIC to prioritise waiting patients.
- All patients less than 18 years of age must be discussed with a Duty Consultant Psychiatrist within 8 hours of admission to the ED.
- Reviews all referrals from the ED team.
- Sources history and patient information from referrers, other health professionals, support people and PSOLIS/ webPSOLIS healthcare record.
- Oversees and coordinates the activity of the ED MHLT Team, in conjunction with ED MHLT Registrar; in particular the MH assessment, monitoring and care of patients in ED who have been referred to the ED MHLT Team.
- Prepares and sends referrals to the appropriate inpatient service (refer [RPBG General Health Admission, Discharge and Transfer SOP](#) and/or [RPBG Mental Health Inpatient Triage to Discharge SOP](#)) including collation of documentation required at point of transfer of care.
- Updates MH information on the ED Information System (EDIS).
- Reviews EDIS tracking screen to identify patients who have been screened by the Triage Nurse or Waiting Room (WR) Nurse as suitable for the PPMHS.
- Assists with supporting patients in acute distress and/or experiencing behavioural disturbances.
- Facilitates patient flow to MH wards and services between 1700-0800hrs.
- Provides clinical leadership for the Mental Health Emergency Centre (MHEC).
- Reviews *Behavioural Observation Form (BOF)* of patients under the governance of the ED MHLT. This may be in conjunction with the ED CNS or delegate to ensure least restrictive measures.
- Assesses ongoing psychiatric care of patients at least once per shift, including expiry date and times of current MHA 2014 forms.
- Documents incidents or hazards in Datix Clinical Incident Management System (CIMS) and/or Combined Hazard or Incident Report (CHoIR). Additionally, completes of Notifiable Incident Reporting to the Office of the Chief Psychiatrist (OCP) as per the [Policy for Reporting of Notifiable Incidents to the Chief Psychiatrist Public MH Services](#).

Psychiatric Liaison Service (PLS) Admitting Registrar (24/7)

- As the delegate of the Duty Consultant Psychiatrist the Psychiatric Admitting Registrar is responsible for MH assessment and management of patients who have been referred to the ED MHLT (working in conjunction with CNS MHL).
- Collaborates with the Consultant in assessing new patients in the MHEC.
- Collaborates and liaises with the CNS MHL to:
 - determine individual workloads and allocate priorities.
 - provide regular updates regarding the management of their allocated caseloads.
 - update the ED team regarding assessments and management plans after every review of the patient.

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- Seeks support from MHEC Registrar, Intern and/or Duty Consultant Psychiatrist at times of high demand or when the CNS MHL is absent.
- Supports the CNS MHL with completion of CIMS, CHoIRs and/or Notifiable Incident Reporting to the OCP for MH patients.
- Supports the CNS MHL in completing referrals for inpatient admissions.
- Provides advice to RPBG ward and service staff on the management of MH patients in their areas, including behavioural management, as required.
- Ensures that the Duty Consultant Psychiatrist is notified of all patients less than 18 years within 8 hours of admission to the ED.

Consultant Psychiatrist (Mon-Sun 0800-1700hrs) and Duty Consultant Psychiatrist (Afterhours on-call 1700-0800hrs)

- Reviews and/or provides advice regarding the management of all high-risk patients escalated by the Psychiatric Registrar, CNS MHL and EPIC.
- Reviews all patients under the MHA 2014 in the RPH ED.
- Consults with the Child and Adolescent Mental Health Service (CAMHS) On Call Consultant Psychiatrist to discuss the assessment and management plan of patients less than 16 years.
- Undertakes advisory and supportive work for the ED MHLT and provides supervision for the Psychiatric Registrar in ED.
- Advocates for patients with the Duty Consultants of other specialties within and outside of MH services, as required.
- Has an active role in the resolution of disagreements regarding clinical issues when escalated by ED staff, the Psychiatry Registrar or the CNS MHL.
- Acts in accordance with the MHA 2014.

ED Triage Nurse (24/7)

- Triage as per [RPBG ED: Triage and Waiting Room SOP](#).
- Identifies and assesses patients whose presentation is affected by MH or behavioural disturbances.
- Collaboratively completes the PPMHS Tool with the ED Waiting Room (WR) Nurse for all patients who present with a primary MH condition and are triaged as an ATS 4 or 5.
- Allocates patients who meet the inclusion criteria on the PPMHS using the Nurse section in the EDIS Clinical Window.
- Contacts PLN via phone and/or CARPS (share role with WR Nurse).
- Advises EPIC of patient on the PPMHS. Considers the assessed risk, degree of patient distress and/or agitation and likelihood that patient will leave before being assessed "Did Not Wait" (DNW).
- Makes a clinical decision regarding the:
 - Appropriate initial location for patient. Allocation to the WR should be assessed on an individual basis. If this location poses a risk to the patient or others, then an alternative location should be sought in liaison with pod leaders.

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- Need to inform, discuss and/or immediately escalate the patient with the ED MHLT and/or escalate to the EPIC or Nursing Shift Coordinator for additional resources, support and/or advice.
- Requirement for Security presence including if searching of patient is required. Refer to [EMHS Chaperone Policy](#) and [EMHS Searching of Patients Belonging and Property Policy](#).
- Liaises with the CNS MHL regarding the above decisions as required.

ED Nursing Shift Coordinator (24/7)

- Liaises with the ED MHLT to arrange an appropriate bed if an admission is likely from 2230hrs to 0700hrs.
- Informs the CNS MHL of significant changes to the patient's presentation, e.g. level of risk (or delegates this task).
- Determines if a nursing special or companion is required in liaison with the pod leader, ED medical team and ED MHLT. The ED Nursing Shift Coordinator ensures request has been documented and rationale has been provided. Refer to [Special and Supportive Observations Policy](#).

ED Medical Team (24/7)

- Reviews patients and determines if MH assessment is required.
- Refers to ED MHLT.
- Completes EBM booking slip.
- Completes *Mental Health Medical Handover DMR template* for patients who are going to be admitted to a MH ward/service. Refer to [ED Medical Handover template in DMR](#) section.
- Develops initial care plan including advice on behaviour management based on relevant risks.
- Responds to any MH emergencies within the ED as required.
- Refer to the [HDWA Safety Planning for Mental Health Consumers Policy and RPBG Inpatient Admissions from the ED Policy](#).

ED Flow CNS (Mon-Fri 0700-2230hrs)

- Liaises with the EMHS Mental Health Patient Flow Coordinator and/or Operations Hub CNM after hours and ED MHLT to arrange an appropriate bed if an admission is likely from 0700hrs to 2230hrs.

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Escalating care due to MH clinician staffing deficits

Should the Psychiatric Admitting Registrar not be available:

- Immediately contact the ED Consultant Psychiatrist and provide details of the time delay and number of patients requiring assessment.
- If further escalation is needed, contact:
 - In hours: On Call Psychiatrist or MH Division Medical Co-Director.
 - Afterhours: Duty Psychiatrist or Medical Executive on Call.

Should the CNS MHL not be available:

- In hours:
 - contact the Coordinator of Nursing (CoN), MH, RPH.
- After hours:
 - For issues regarding patient flow, contact the Bentley Health Service (BHS) MH CNS.
 - For issues regarding PSOLIS/ web PSOLIS information requests, contact the MHEC Shift Coordinator.
 - For any other clinical or patient flow issues, contact the Operations Hub CNM who can liaise with the Senior Nurse on Call (SNoC) if required.

Should the CNS MHL and Psychiatry Admitting Registrar not be available:

In addition to the above, the CNS MHL or Registrar on shift immediately prior to known staff deficit to:

- Notify community teams of Registrar and/or CNS MHL deficit.
- Notify Operations Hub CNM for escalation to State Health Operations Centre (SHOC).
- Advise CMHS to avoid, where possible, voluntary referrals for admission to RPH ED. Notify the Mental Health Unit (MHU), Ward 2K and MHEC CN and/or Shift Coordinator for awareness of escalation pathways where there is no MH Division Medical Practitioner on site to undertake requirements of the MHA 2014.
- Notify Safety All Hours For Everyone (SAFE) Team of potential increased need for support to MHU, Ward 2K and MHEC in the absence of CNS MHL or Registrar.
- Contact the Bentley Health Service (BHS) After Hours (AH) MH CNS for possible support over the phone required for RPH ED, MHEC, MHU, Ward 2K.
- Complete end of shift huddle with EPIC, ED CNS, MH On Call Consultant for comprehensive handover of current patient plan and potential escalation required during the shift.

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Ongoing management of mental health presentations in ED

- ED EPIC is the point of contact and will liaise directly with MH Consultant on Call for involuntary patient referrals received by RPH.
- MH Consultant Psychiatrist on call will be advised of admissions and provide support and/or guidance by phone but may be required to attend on site for review of patients.
- If ED is under or proceeds to a code yellow MH EPIC will liaise with Operations Hub CNM to load level between sites by requesting CMHS teams to form involuntary patients to other sites. EPIC will need to liaise with MH Consultant Psychiatrist On Call and Operations Hub CNM to advise of this escalation.

PSOLIS/ webPSOLIS

The ED MHLT has access to PSOLIS/ webPSOLIS. ED staff can request PSOLIS alerts for patients who present to ED. If the ED MHLT note a patient present in the ED with PSOLIS alerts, they are to inform the ED team and provide a printed copy of the relevant alerts for the patient's healthcare record. Clinical comments can be added to EDIS if appropriate.

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Medical assessment in the ED

Medical assessment is a snapshot in time where there is a medical evaluation of patients who present to the ED with a MH crisis before either discharge or admission.

This should include if possible:

- A clear and reliable patient history including collateral history from the support person(s) if available.
- MH examination.
- Risk assessment:
 - Risk of suicide and/or self-harm.
 - Absconding or left without notice.
 - Underlying organic issue.
 - Vital signs.
 - Physical examination (where appropriate).
- Medical assessment to assist in determining if presentation is due to underlying physical health condition.
- High index of organic cause of MH presentations in the following context:
 - New presentations.
 - Older persons greater than 65 years of age or.
 - greater than 50 years of age in Aboriginal, Torres Strait Islander, Maori and Pacific people.
 - Abnormal vital signs.
 - Atypical symptoms (e.g. visual hallucinations).
- Patients who present with alcohol and drug intoxication.
 - Patients who present with a breathalyser alcohol level (BAL) of greater than 0.05% and look clinically intoxicated would benefit from observation in Emergency Medicine Ward (EMW) to sober up prior to determining if they would benefit from a MH review.
 - Acute psychosis secondary to methamphetamine use, which does not resolve after a period of sedation greater than 12 hours require MH assessment as it may be an exacerbation of an underlying psychiatric illness.
 - Routine urine drug screen (UDS) does NOT have a role in the ED. It has multiple limitations and should only be used as a tool to aid diagnosis.-Absence of completed UDS should not be a barrier for assessment by the Psychiatric Registrar or ED MHLT.
- Determine when patient is able to be interviewed, to enable:
 - Initiation of MH assessments:
 - These psychiatric assessments can occur in parallel with medical assessments prior to definitive stabilisation
 - BAL of 0.05% is simply the legal limit to drive, it is not an indication of level of clinical intoxication. Patients are considered able to be interviewed if they are clinically not intoxicated and ideally if their BAL is less than 0.05%. A blood alcohol level is not required.
- ED medical team to document time medical assessment completed to indicate the patient is medically stable and that their MH condition is unlikely due to an underlying physical health condition. They are therefore appropriate for admission/ discharge to psychiatric services. The use of the term 'Medical Clearance' is not recommended for use due to being ambiguous, misleading and unhelpful.

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ED medical handover template in the Digital Medical Record (DMR)

This template summarises the medical assessment to the MHLT and is a snapshot in time. Medical status of a patient can change over time the longer the patient remains within the ED. Concerns about medical stability to be referred to ED Medical Team whilst the patient is in ED.

Fields in the template include:

- Primary reason for ED presentation
- Active medical issues
- Active behavioural issues
- Signs of organic disease Yes/No
- ECG and/or Bloods Yes/No
- Impaired mobility and/or falls risk Yes/No
- Chemical restraint Yes/No
- Physical restraint Yes/No
- Active recreational drug and alcohol issues Yes/No
- Follow-up requirements
- Summary: e.g. Medically stable for admission to a psychiatric unit.

Co-existing urgent management of both physical and MH conditions

Senior ED clinicians and the MHLT to liaise and develop a collaborative plan with the priorities identified.

A determination of the acute medical needs versus the stable chronic medical needs to be attended and the appropriate destination where the patient's requirements are best met agreed upon.

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MH assessment

- All referrals to the ED MHLT to have documented MH assessment. In the event that a referral does not result in an assessment the minimum expectations include:
 - In person discussion with the referrer.
 - PSOLIS/ webPSOLIS history check.
 - Sighting of the patient.
 - Documentation in healthcare record of rationale leading to decision to not assess the patient.
- As part of the MH Assessment, the ED MHLT:
 - completes a comprehensive MH and risk assessment including an MSE and documents on the *SSCD Triage form* and *RAMP* as per [WA Health SSCD for Mental Health Services Policy](#).
 - utilises interpreters, Aboriginal Liaison Service and/or Aboriginal MH workers and/or traditional healers and/or elders, to facilitate patient assessment when required and available.
 - undertakes joint reviews with the Medical Officer and the CNS MHL as the preferred assessment modality, where possible.
 - determine the treatment plan e.g. admission to RPBG ward or service, transfer to another MH service or discharge from the ED with appropriate community follow up.
 - complete transfer to other MH services in accordance with the [EMHS Mental Health Bed Access, Capacity, and Escalation Framework](#).
- Complete a Triage and RAMP in PSOLIS/ webPSOLIS, printout documentation for non-admitted patients from areas including WR, Ambulance Bay (ABay), RAMP, triage corridor (TCORR) if the following steps have occurred.
 - Patient risk assessed as being suitable for assessment prior to being brought into ED.
 - Patient management discussed with EPIC and Triage Team Leader to ensure there is appropriate agreement of plan post review.
- Transfer of care to the ED MHLT occurs at point of assessment by ED MHLT following referral from the ED.
- The ED MHLT team are to provide all patients reviewed and discharged from ED with a clear *Treatment, Support and Discharge Plan* and *Care Transfer Summary* and document the plan in the patient's healthcare record, including any referrals or follow up arrangements.
- The ED MHLT team are to provide the patient and support people with written advice including contact details of the relevant CMHS, timelines for follow up, and 24-hour MH emergency contacts.
- Where an active CMHS patient with MH presentation is seen in the ED, the ED MHLT team will advise the treating CMHS team of the presentation via phone during business hours and via email with link to assessment after hours.
- Where a patient is admitted to a general inpatient bed but requires further MH assessment, the CNS MHL or Psychiatry Registrar will request the admitting specialty submit an eReferral to the inpatient Consultation Liaison (C-L) team.

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Escalation to ED MHLT consultant psychiatrist

Patients referred to the ED MHLT for review are to be discussed with the ED Psychiatry Registrar before leaving the ED if:

- Assessed as high risk - including patients less than 16 years and patients who are perinatal or postnatal.
- Aboriginal patients, patients from culturally and linguistically diverse (CALD) backgrounds and patients at high risk of suicide and/or self-harm.
- Medically complex.
- Referred to the ED for MH input by a Medical Practitioner e.g. General Practitioner (GP).
- Subject to the MHA 2014.
- Planning discharge for patients greater than 65 years.
- Presentations to any ED within 7 days of discharge from any MH inpatient unit.

All patients less than 18 years of age must be discussed with a Duty Consultant Psychiatrist within 8 hours of admission to the ED.

Escalate to the Consultant Psychiatrist where clinically indicated. Refer to the [RPBG Escalation to Consultants Policy](#).

Management of patients presenting with behaviours of concern

- Not all ED patients who present with disturbed behaviour require specialist MH referral. The ED clinician should use clinical judgement to determine the underlying cause which may not necessarily be psychiatric.
- For further information refer to the [Behaviours of Concern \(Preventing, Recognising and Responding\) CPS](#).

All staff must immediately activate a “Code Black – Personal Threat” if there is a threat to self or others using personal or fixed duress alarms, or by calling 55.

Missing or AWOL patients

- For factors that may contribute to the risk of patients leaving ED without notice (i.e. AWOL (absconding)) refer to the [EMHS Mental Health Missing Persons/Absent Without Leave Policy](#).
- Patients assessed as at risk of leaving without notice or AWOL are:
 - Allocated to an area in the ED that enables appropriate observations.
 - Considered for 1:1 special.
 - Prioritised for assessment by the ED MHLT.

AWOL Patients not yet Referred to the ED MHLT

- Patients who have been triaged but not yet referred to the ED MHLT team and who are AWOL or left without notice are managed by ED staff as per the [EMHS Management and Review of 'Did Not Wait' Patients that Present to Emergency Departments Policy](#) and [EMHS Mental Health Missing Persons/Absent Without Leave Policy](#).
- If a patient goes missing from the ED, staff to assess the risk the patient poses to self and others. The assessment should be used to inform notification and reporting processes, and guide the urgency of response required by ED staff.
- Ensure EPIC and ED Shift Coordinator awareness, and document in patient healthcare record.
- All clinical staff should seek advice from the ED MHLT team regarding risk and appropriate action as required. Timely notification to Operations Hub CNM is to occur where there is a risk of harm to self and/or others.

AWOL patients referred to the ED MHLT Team but not assessed

- The staff member who notices a patient leave without notice / AWOL informs the ED MHLT of the patient's departure as soon as possible.
- The CNS MHL or Psychiatry Registrar will review all available information in PSOLIS/ webPSOLIS to provide any collateral information to the ED clinical team.
- The ED clinical team will determine interventions based on the patient's assessed level of risk and may include:
 - No further action.
 - Telephoning the patient to assess risks and then determining further action.
 - Telephoning support people to obtain further information and then determining further action.
 - Requesting assessment by the local CMHS team.
 - Notifying the WA police (WAPOL) and discuss and/or recommending interventions by WAPOL.
- Refer to [EMHS Missing or Suspected Missing In-Patients – General Policy](#) Document the management plan and complete a Datix CIMS and/or Notifiable Incident Reporting to the Office of the Chief Psychiatrist (OCP) for MH patients, as appropriate. Refer to [EMHS Notifications requirements under the WA Mental Health Act 2014 Policy](#).

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AWOL patients assessed by MH and requiring a MH admission

- The CNS MHL reviews patient information in the PSOLIS/ webPSOLIS. Action and/or interventions are to be determined by the patient's assessed risk level prior to AWOL or leaving without notice (absconding) and include:
 - Reviewing level of risk documented on the RAMP.
 - Contacting the patient by phone.
 - Contacting support people by phone.
 - Informing the Duty Psychiatrist.
 - Informing the referrer.
 - Informing Security and requesting a search of the health service site.
- For active MH patients request follow up by:
 - The relevant Community Treatment Team during working hours.
 - The relevant catchment area Assessment Treating Team (ATT) after hours (where practicable).
- For a referred patient who is detained or an involuntary detained patient who is AWOL complete a Datix CIMS and Notification to OCP. Refer to [Policy for Reporting of Notifiable Incidents to the Chief Psychiatrist – Public Mental Health Services](#) via the [Office of the Chief Psychiatrist Western Australia website](#).
 - Request WAPOL assistance to apprehend and return the patient to ED for assessment if risks are assessed as high or if the patient is on an MHA 2014 Form 1A & 3A (see MHA 2014 Application in ED Section 7) and contact is not possible.
 - Document the actions and/or plan in the patient's healthcare record.
 - Escalate to or notify:
 - In hours: Consultant, NUM, EPIC, Psychiatrist.
 - After hours: Operations Hub CNM.
 - Refer to [EMHS Mental Health Missing Person/ Absent Without Leave Policy](#), [EMHS Metal Health Missing Persons/ Absent Without Leave Guideline](#).

All returning AWOL patients are to be reassessed by ED MHLT. Patient risk assessment and risk mitigation strategies to be reviewed and plan updated as required.

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MHA 2014 application in ED

Medical Practitioner/ AMHP are to:

- Complete **Form 1A** – Referral for Examination by a Psychiatrist following a face-to-face patient assessment within the preceding 48 hours and must determine that:
 - The patient is experiencing a mental illness for which they require treatment.
 - and/or the patient's mental illness poses a significant risk either to the patient's health and/or safety or that of another person.
 - and/or the patient does not demonstrate that they have the capacity to make treatment decisions.
 - There is not a less restrictive way to provide the required treatment.
- A Form 1A is valid for 72 hours and may be written to:
 - An authorised hospital (either RPH, BHS or another internal or external site e.g., in ED for consideration of an involuntary inpatient in a general hospital).
 - If the form 1A is written to the ED, the patient is immediately received when the referred patient enters the ED.
 - Complete the section at the bottom of Form 1A to identify the commencement of the 24-hour period during which the patient must be examined by a Psychiatrist.
 - Ensure the patient is examined by a Consultant Psychiatrist within 24 hours.
 - External to the ED (Other services e.g., ED, MH clinic or general hospital).
 - **Revoke the referral** – where it is determined that the referred patient no longer requires involuntary treatment, complete the revocation section of Form 1A.
 - A **Form 1B Variation of Referral** if it is required to change the place where the examination is to occur or the service the patient is referred to is unable to receive the patient.
- Complete a **Form 3A** (Detention Order) if the patient needs to be detained to enable them to be taken to the examination - valid for 24 hours. Provide a copy of Form 3A to the patient and ensure patient's support people are notified. **This is a notifiable event.**
 - If a patient who is on a Form 3A goes AWOL from ED, complete a **Form 7D Apprehension and Return Order** valid for 14 days to enable WAPOL or a staff member of a MH service to apprehend the patient and return them to the place where they had been.
 - If it is determined that the patient needs to be detained for further periods of 24 hours, complete **Continuation of Detention Order Form 3B**. Notify the patient's support people **This is a notifiable event.**
- Determine patient transport requirements for the examination.
 - a) Preferably, if the patient can travel safely with their support people or a health professional or community staff member.
 - b) Complete a **Transport Order Form 4A** to authorise a transport officer or WAPOL to apprehend the patient and transport them; valid for 72 hours.
 - Provide a copy of Form 4A to the patient and notify the patient's support people. **This is a notifiable event.**
- See [Approved Forms for Public Services on the OCP website](#) which are preferably to be completed directly on PSOLIS/ webPSOLIS, printed and stored on the patient's healthcare record. If forms are unavailable or staff are unable to access PSOLIS/ webPSOLIS, staff are to use writable PDFs or a print hard copy. For further guidance, refer to [OCP MHA 2014 Information for Referring Practitioners](#) and [EMHS Notifications requirements under the WA Mental Health Act 2014 Policy](#).

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Compliance monitoring

Compliance for this document will be monitored in the following ways:

By the MH Division via:

- Clinical incident trending through CIMS, CHoIR and OCP notifications
- ED MH AWOL or Leave without notice patient trending
- Patient feedback and/or complaints tracking
- Percentage of MH ED attendances with a length of episode greater than 24 hours.

By the Hospital Logistics and Acute Access (HoLAA) Division via:

- Clinical incident trending through CIMS and CHoIR
- Patient feedback and/or complaints tracking
- Western Australian Emergency Access Targets (WEAT)

Facilitator

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Bibliography

1. Department of Health National Framework for Recovery Oriented Mental Health Services: <http://www.health.gov.au/internet/main/publishing.nsf/content/mental-pubs-n-recovfra>
2. Charter of Mental Health Care Principles (MHA 2014)
3. Australian Commission Safety & Quality, Available from: <https://www.safetyandquality.gov.au/our-work/partnering-consumers/person-centred-care>
4. Trauma-informed care and practice Available from: <https://www.mhcc.org.au/project/trauma-informed-care-and-practice-ticp/>

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Related policies, practice standards, clinical guidelines

[RPBG Policy Hub](#)

- ED Quick Assessment and Care Operational Management SOP
- Recognising and Responding to Acute Deterioration in Mental State, Cognition and Behaviour SOP
- Recognising and Responding to Acute Deterioration Policy
- Behaviours of Concern (Preventing, Recognising and Responding) CPS
- Special and Supportive Observations Policy

[EMHS Policy Hub](#)

- [EMHS Mental Health Bed Access, Capacity, and Escalation Framework.](#)
- [EMHS Notifications requirements under the WA Mental Health Act 2014 Policy.](#)

[Department of Health Policy Hub](#)

- Recognising and Responding to Acute Deterioration Policy
- [Mental Health Consumers who are Missing or Absent Without Leave Policy](#)
- [Safety-Planning-Procedures-for-Mental-Health-Consumers \(pdf 149KB\)](#)
- [Principles-and-Best-Practice-for-the-Care-of-People-Who-May-Be-Suicidal \(pdf 360KB\)](#)
- [Principles for the Care of People Who May Be at Risk of Violent or Aggressive Behaviour](#)
- [Policy for Reporting of Notifiable Incidents to the Chief Psychiatrist – Public Mental Health Services](#)

Other

- MHA 2014

Related National Standards

ACSQHC NSQHS Standards 2nd Edition (2021)

Standard 1: Clinical Governance

Standard 2: Partnering with Consumers

Standard 5: Comprehensive Care

Standard 8: Recognising and Responding to Acute Deterioration

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